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Dayal Prakash Jaiswal

25 Year(s) · Male

Reported on

January 29, 2026

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Dayal Prakash Jaiswal

25 Year(s)/Male

Ref. by :

Partner : Elevate now

Report Ref. ID : NOA6055546

Patient ID : OHPXSBGO1914626

Collected : 29/01/2026 08:45 AM

Received : 29/01/2026 11:01 AM

Reported : 29/01/2026 03:55 PM



MC-6357



| Test | Results | Units | Biological Reference |
|------|---------|-------|----------------------|
|------|---------|-------|----------------------|

**BIOCHEMISTRY****Glucose, Fasting**

Fluoride Plasma, Glucose Oxidase-Peroxidase (GOD-POD)

92

mg/dL

70 - 99

**Amylase**

Serum, Twopoint-Rate-Dyed Amylopectin

62

U/L

30 - 110

**Clinical Significance:**

Serum amylase rises within 6-48hrs of onset of acute pancreatitis but is not proportional to the severity. Usually returns to normal in 3-5 days, values persisting longer than this period is suggestive of necrosis of pancreas or pseudocyst of pancreas. Non-pancreatic causes of hypeamylasemia includes inflammatory salivary lesions, perforated peptic ulcer, intestinal obstruction and infarction, aortic aneurysm, peritonitis.

**Lipase**

Serum, Twopoint-Rate with Colipase

117

U/L

23 - 300

**Clinical Significance:**

Lipase is elevated with amylase in acute pancreatitis and remains high for 7-10 days. Combined use of amylase and lipase is effective in ruling out acute pancreatitis.

Significant elevations of serum lipase not caused by pancreatitis: a systematic review - <https://pmc.ncbi.nlm.nih.gov/articles/PMC4299384/>

Elevated lipase levels, like raised amylase levels, can occur in both asymptomatic patients and differing medical and surgical conditions other than pancreatitis. Reduced clearance with renal impairment or macrolipase formation, other intra-abdominal pathologies, diabetes, drugs and infection can also be responsible for lipase levels of three or more times the upper limit of normal.

**Lipid Profile | Serum****Cholesterol, Total**

Cholesterol Esterase/Cholesterol Oxidase/Peroxidase

124

mg/dL

&lt; 200

**Triglycerides**

Cholesterol Oxidase

82

mg/dL

&lt; 150

**High-Density Lipoprotein (HDL)****Cholesterol**

Cholesterol Esterase/Cholesterol Oxidase/Peroxidase

51

mg/dL

&gt; 50

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| Non-High Density Lipoprotein (Non-HDL) Cholesterol<br>Calculated               | 73       | mg/dL | < 130                |
| Low-Density Lipoprotein (LDL) Cholesterol<br>Calculated                        | 57       | mg/dL | < 100                |
| Very Low-Density Lipoprotein (VLDL) Cholesterol<br>Calculated                  | 16       | mg/dL | < 30                 |
| Cholesterol/High Density Lipoprotein (HDL) Ratio<br>Calculated                 | <u>2</u> |       | 3.3 - 4.4            |
| Low-Density Lipoprotein/High-Density Lipoprotein (LDL/HDL) Ratio<br>Calculated | 1.1      |       | 0.5 - 3              |
| High-Density Lipoprotein/Low-Density Lipoprotein (HDL/LDL) Ratio<br>Calculated | 0.9      | Ratio | > 0.4                |

| Remarks       | Total Cholesterol (mg/dL) | Triglycerides (mg/dL) | LDL Cholesterol (mg/dL) |
|---------------|---------------------------|-----------------------|-------------------------|
| Optimal       | <200                      | <150                  | <100                    |
| Above Optimal | -                         | -                     | 100-129                 |
| Borderline    | 200-239                   | 150-199               | 130-159                 |
| High          | ≥ 240                     | 200-499               | 160-189                 |
| Very High     | -                         | ≥ 500                 | ≥ 190                   |

Lipid profile is a group test consisting of various lipids. Lipid profiles are generally collected with overnight fasting. However, recent guidelines have recommended non fasting samples for lipid profile for assessment of cardiovascular risk. The details for the study can be checked at <https://jamanetwork.com/journals/jamainternalmedicine/fullarticle/2733560>

In certain instances measurements in the same patient can show physiological and analytical variations. In such cases three serial samples at an interval of 1 week each are recommended for Total cholesterol, TG, HDL and LDL.

**Cholesterol levels** are increased in primary hypercholesterolemia; secondary hyperlipoproteinemia, including nephrotic syndrome; primary biliary cirrhosis; hypothyroidism; and in some cases, diabetes mellitus. Low cholesterol levels may be found in malnutrition, malabsorption, advanced malignancy, and hyperthyroidism.

**Triglyceride levels** are used in the diagnosis and treatment of patients with diabetes mellitus, nephrosis, liver obstruction, other diseases involving lipid metabolism, or various endocrine disorders.

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**High Density Lipoprotein (HDL) cholesterol levels** is used to evaluate the risk of developing coronary heart disease (CHD). The risk of CHD increases with lower HDL cholesterol concentrations.

**LDL (low-density lipoprotein) cholesterol level**, sometimes called "bad" cholesterol, makes up most of our body's cholesterol. High levels of LDL cholesterol raise your risk for heart disease and stroke.

**Very-low-density lipoprotein (VLDL) cholesterol** is produced in the liver and released into the bloodstream to supply body tissues with triglycerides. High levels of VLDL cholesterol have been associated with the development of plaque deposits on artery walls, which narrow the passage and restrict blood flow.

**Liver Function Test (LFT) | Serum**

|                                                                                                  |      |       |           |
|--------------------------------------------------------------------------------------------------|------|-------|-----------|
| <b>Bilirubin, Total</b><br>Diazo Method                                                          | 0.80 | mg/dL | 0.2 - 1.3 |
| <b>Bilirubin, Direct</b><br>Calculated                                                           | 0.2  | mg/dL | 0 - 0.3   |
| <b>Bilirubin, Indirect</b><br>Reflectance Spectrophotometry                                      | 0.60 | mg/dL | 0.1 - 1.1 |
| <b>Aspartate Aminotransferase (AST)</b><br>Multipoint-Rate/UV with Pyridoxal-5-Phosphate (P-5-P) | 32   | U/L   | 17 - 49   |
| <b>Alanine Transaminase (ALT)</b><br>LDH, UV Kinetic                                             | 28   | U/L   | <50       |
| <b>Aspartate<br/>Aminotransferase/Alanine<br/>Transaminase (AST/ALT) Ratio</b><br>Calculated     | 1.1  |       | 0.7 - 1.4 |
| <b>Alkaline Phosphatase (ALP)</b><br>Multipoint-Rate/UV with Pyridoxal-5-Phosphate (P-5-P)       | 60   | U/L   | 38 - 126  |
| <b>Gamma-Glutamyl Transpeptidase (GGT)</b><br>SZAZ Carboxylated Substrate                        | 30   | U/L   | 15 - 73   |
| <b>Protein</b><br>Biuret                                                                         | 8.0  | g/dL  | 6 - 8.3   |
| <b>Albumin</b><br>Bromo-Cresol Green                                                             | 4.5  | g/dL  | 3.5 - 5   |
| <b>Globulin</b><br>Calculated                                                                    | 3.5  | g/dL  | 2.3 - 3.5 |

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|--------------------------------------------|---------|-------|----------------------|
| Albumin/Globulin (A/G) Ratio<br>Calculated | 1.3     | Ratio | 0.8 - 2              |

In certain individuals, **total bilirubin** up to 2.0 mg/dl is considered normal. High bilirubin values can be due to jaundice.

**Total bilirubin** is invariably increased in jaundice. Causes of jaundice are prehepatic, resulting from various hemolytic diseases; hepatic, resulting from hepatocellular injury or obstruction; and posthepatic, resulting from obstruction of the hepatic or common bile ducts.

Increased **direct bilirubin** levels can occur in hepatobiliary disorders, including intrahepatic and extrahepatic biliary tree obstruction, liver cell damage, Dubin-Johnson syndrome, and Rotor syndrome.

High **indirect bilirubin** levels can occur in hemolytic disorders, Gilbert's syndrome, Crigler-Najjar syndrome, neonatal jaundice, and ineffective erythropoiesis.

High **Aspartate Aminotransferase** values can occur in Myocardial infarction, pulmonary emboli, skeletal muscle trauma, alcoholic cirrhosis, viral hepatitis, or drug-induced hepatitis.

Elevated **Alanine Aminotransferase** levels are seen in liver cell necrosis, hepatitis, hepatic cirrhosis, liver tumours, obstructive jaundice, Reye's syndrome, extensive trauma to skeletal muscle, myositis, myocarditis, or myocardial infarction.

High **alkaline phosphatase** levels can be due to primary and secondary hyperparathyroidism, Paget's disease of bone, carcinoma metastatic to the bone, osteogenic sarcoma, Hodgkin's disease, Hepatobiliary diseases involving cholestasis, inflammation, or cirrhosis.

**ALP** levels can also be elevated in fever and increased bone metabolism(e.g., in adolescents and during the healing of a fracture), in renal infarction and failure and in pregnancy complications.

Low **ALP** levels may occasionally be seen in hypothyroidism.

**Gamma-glutamyl transferase (GGT)** is a sensitive indicator of hepatobiliary disease. It is useful in the diagnosis of obstructive jaundice and chronic alcoholic liver disease, in the follow-up of chronic alcoholics undergoing treatment, and in the detection of hepatotoxicity. GGT is more responsive to biliary obstruction than AST, ALT, or ALP.

Total **protein** levels can be used to evaluate nutritional status.

High **protein** concentrations can be due to dehydration, Waldenström's macroglobulinemia, multiple myeloma, hyperglobulinemia, granulomatous, and some tropical diseases.

Low **protein** concentrations can be due to pregnancy, excessive intravenous fluid administration, cirrhosis or other liver diseases, chronic alcoholism, heart failure, nephrotic syndrome, glomerulonephritis, neoplasia, protein-losing enteropathies, malabsorption, and severe malnutrition.

Increased **albumin** levels may indicate dehydration or hyperinfusion with albumin.

Decreased **albumin** levels are found in rapid or over-hydration, severe malnutrition and malabsorption, severe diffuse liver necrosis, chronic active hepatitis, and neoplasia.

**Albumin** is commonly reduced in chronic alcoholism, pregnancy, renal protein loss, thyroid dysfunction, peptic ulcer disease, and chronic inflammatory diseases.

**Globulin** includes carrier proteins, enzymes, complement, and immunoglobulins. Most of these are synthesised in the liver, although immunoglobulins are synthesised by plasma cells.

Increased **globulin** level usually results from an increase in immunoglobulins.

Malnutrition and congenital immune deficiency can decrease **globulin** levels due to decreased synthesis. Nephrotic syndrome can cause decreased globulin levels due to protein loss through the kidney.

**AST/ALT Ratio** > 2:1 (AST is two times higher than ALT) is indicative of alcoholic liver disease.

**AST/ALT Ratio** < 1:1 (ALT is higher than AST) indicates non-alcoholic fatty liver disease.

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|----------------------------------------------------------------------------------------------------|---------|---------------------------|-----------------------------------------------------------------------------------------------------------------------------------|
| Kidney Function Test (KFT) Mini   Serum                                                            |         |                           |                                                                                                                                   |
| Urea<br>Urease                                                                                     | 28      | mg/dL                     | 19 - 43                                                                                                                           |
| Creatinine<br>Twopoint-Rate-Creatinine Aminohydrolase                                              | 1.0     | mg/dL                     | 0.66 - 1.25                                                                                                                       |
| Blood Urea Nitrogen (BUN)<br>Calculated                                                            | 13.1    | mg/dL                     | 6 - 20                                                                                                                            |
| Blood Urea Nitrogen (BUN)/Creatinine Ratio<br>Calculated                                           | 13.10   | Ratio                     | 10 - 20                                                                                                                           |
| Uric Acid<br>Uricase                                                                               | 8.1     | mg/dL                     | 3.5 - 8.5                                                                                                                         |
| Calcium<br>Arsenazo Method                                                                         | 9.3     | mg/dL                     | 8.4 - 10.2                                                                                                                        |
| Estimated Glomerular Filtration Rate (eGFR)<br>Twopoint-Rate-Creatinine Aminohydrolase/Calculation | 107     | ml/min/1.73m <sup>2</sup> | Normal: ⇒ 90<br>Mild decrease: 60-89<br>Mild moderate decrease: 45-59<br>Severe decrease: 15-29<br>End stage kidney disease: < 15 |

High **urea** and **BUN** levels are suggestive of poor kidney function due to acute or chronic kidney diseases, decreased blood flow to the kidneys as in congestive heart failure, shock, stress, recent heart attack or severe burns, bleeding from the gastrointestinal tract, conditions that obstruct urine flow or dehydration.

Low **urea** and **BUN** levels are uncommon and are not usually a cause for concern. They can be seen in severe liver disease or malnutrition but are not used to diagnose or monitor these conditions. Low urea levels are also seen in normal pregnancy.

**Creatinine** is elevated in kidney disease, damage, infection, urinary tract obstruction, reduced blood flow to the kidneys in case of shock, congestive heart failure, complications of diabetes.

High levels of **uric acid** are seen in kidney disease, pre-eclampsia, purine-rich food, alcoholism, and side effects of cancer treatment.

Low **calcium** levels may be due to hypoparathyroidism, kidney failure, pancreatitis, malnutrition, or a disorder in calcium absorption.

High **calcium** levels may be due to hyperparathyroidism, hyperthyroidism, sarcoidosis, drugs like diuretics, and excessive calcium supplementation.

High **phosphorus** levels can be due to dehydration, hypoparathyroidism, hypervitaminosis D, metastases to bone, sarcoidosis, pulmonary embolism, renal failure, or diabetes mellitus with ketosis.

Low **phosphorus** levels can be caused by hyperparathyroidism, high calcium levels, sepsis, vitamin D deficiency, renal tubular disorders, chronic hemodialysis, vomiting, or occasionally decreased dietary phosphate intake.

Chronic Kidney Disease often has no symptoms until the later stages. So, reliable estimates of GFR are important for identifying the disease as early as possible.

Factors that can affect **eGFR** include pregnancy, being over the age of 70, unusual muscle mass, cirrhosis, nephrotic syndrome, a past solid organ transplant, and some medications.

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| Test                                                                         | Results | Units | Biological Reference                                      |
|------------------------------------------------------------------------------|---------|-------|-----------------------------------------------------------|
| Glycosylated Haemoglobin (HbA1c)   Whole Blood EDTA                          |         |       |                                                           |
| Glycated Hemoglobin (HbA1C)<br>High-Performance Liquid Chromatography (HPLC) | 5.6     | %     | Normal: < 5.7<br>Pre-Diabetes: 5.7-6.4<br>Diabetes: ⇒ 6.5 |
| Mean Blood Glucose<br>Calculated                                             | 114     | mg/dL | < 117                                                     |

**HbA1C** is used to monitor fluctuations in blood glucose concentration in the past 8 to 12 week's period. The reference interval defined as per American Diabetes Association guidelines 2016:

- Less than 5.7%: Non Diabetic
- 5.7 to 6.4%: at increased risk of developing diabetes in the future
- More than 6.5%: Diabetic
- Therapeutic glycemc target
  - Adults: less than 7%
  - Children with Type 1 diabetes: less than 7%
- Pregnant diabetic patients: less than 6.5%

Note: Targets may be individualized based on: Age/life expectancy, Comorbid conditions, Diabetes duration, Hypoglycemia status, Individual patient considerations

Reference: American Diabetes Association. Standards of medical care in diabetes - 2021.

**Mean Blood Glucose** is average Blood glucose which directly correlates with A1C, reported in the same units as blood sugar levels (mg/dl). Thus it reflects the average glucose concentration in the past 8 to 12 weeks period. This should not be compared with Fasting or Post prandial or random blood sugar which measures glucose concentration at that point of time of testing.

Approved By

Dr. Shivani Tiwari

MBBS, DCP (Pathology)  
Pathologist

Dayal Prakash Jaiswal

25 Year(s)/Male

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| Test                                                                    | Results      | Units | Biological Reference |
|-------------------------------------------------------------------------|--------------|-------|----------------------|
| High Sensitivity C-Reactive Protein (HSCR)<br>Serum, Immunoturbidimetry | <b>4.228</b> | mg/L  | Refer below          |

## Clinical Significance:

| hsCRP (mg/L) | Cardiovascular Risk                                                |
|--------------|--------------------------------------------------------------------|
| <1.0         | Low                                                                |
| 1-3          | Average                                                            |
| 3.1-10       | High                                                               |
| >10          | Persistent elevation may represent non-cardiovascular inflammation |

High sensitive CRP (hsCRP) assay is used as an aid in the identification and stratification of individuals at risk for future cardiovascular disease.

Measurement should be done twice (averaging results) optimally two weeks apart, on metabolically stable patients, and should be compared to previous values.

Testing should not be performed while there is indication of infection, systemic inflammation, or trauma.

Approved By

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MBBS, DCP (Pathology)  
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| Test | Results | Units | Biological Reference |
|------|---------|-------|----------------------|
|------|---------|-------|----------------------|

## HAEMATOLOGY

## Complete Blood Count (CBC) | Whole Blood EDTA

|                                                                   |             |                       |                                                                  |
|-------------------------------------------------------------------|-------------|-----------------------|------------------------------------------------------------------|
| Red Blood Cells (RBC) Count<br>DC Impedance Method                | <b>4.21</b> | mill/mm <sup>3</sup>  | 4.5 - 5.5                                                        |
| Hemoglobin (Hb)<br>Cyanide-free SLS method                        | <b>12.9</b> | g/dL                  | 13 - 17                                                          |
| Hematocrit (HCT)   Packed Cell<br>Volume (PCV)<br>Calculated      | <b>38.3</b> | %                     | 40 - 50                                                          |
| Mean Corpuscular Volume (MCV)<br>Calculated                       | 90.9        | fL                    | 83 - 101                                                         |
| Mean Corpuscular Hemoglobin<br>(MCH)<br>Calculated                | 30.6        | pg                    | 27 - 32                                                          |
| Mean Corpuscular Hemoglobin<br>Concentration (MCHC)<br>Calculated | 33.7        | g/dL                  | 31.5 - 34.5                                                      |
| Red Cell Distribution Width (RDW) CV<br>Calculated                | 13.9        | %                     | 11.6 - 14                                                        |
| Mentzer Index<br>Calculated                                       | 22.0        | Index Value           | Beta Thalassemia trait: < 14<br>Iron deficiency anemia: >= 14    |
| Sehgal Index<br>Calculated                                        | 1963.0      | Index Value           | Beta Thalassemia trait: < 972<br>Iron deficiency anaemia: >= 972 |
| Total White Blood Cell Count (TC)<br>Flow Cytometry               | 7200        | cells/mm <sup>3</sup> | 4000 - 10000                                                     |
| Differential Count                                                |             |                       |                                                                  |
| Neutrophils<br>Flow Cytometry                                     | 57.5        | %                     | 40 - 80                                                          |
| Lymphocytes<br>Flow Cytometry                                     | 31.1        | %                     | 20 - 40                                                          |
| Monocytes<br>Flow Cytometry                                       | 7.6         | %                     | 2 - 10                                                           |

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| Test                                                                                                 | Results      | Units               | Biological Reference |
|------------------------------------------------------------------------------------------------------|--------------|---------------------|----------------------|
| Eosinophils<br>Flow Cytometry                                                                        | 3.4          | %                   | 1 - 6                |
| Basophils<br>Flow Cytometry                                                                          | 0.4          | %                   | 0 - 2                |
| Absolute Neutrophil Count (ANC)<br>Calculated                                                        | 4140         | /mm <sup>3</sup>    | 2000 - 7000          |
| Absolute Lymphocyte Count (ALC)<br>Calculated                                                        | 2239         | /mm <sup>3</sup>    | 1000 - 3000          |
| Absolute Monocyte Count (AMC)<br>Calculated                                                          | 547          | /mm <sup>3</sup>    | 200 - 1000           |
| Absolute Eosinophil Count (AEC)<br>Calculated                                                        | 245          | /mm <sup>3</sup>    | 20 - 500             |
| Absolute Basophil Count (ABC)<br>Calculated                                                          | 29           | /mm <sup>3</sup>    | 0 - 100              |
| Neutrophil Lymphocyte Ratio (NLR)<br>Calculated                                                      | 2.0          | Ratio               | 1 - 3                |
| Platelet Count<br>DC Impedance Method                                                                | <b>139</b>   | 10 <sup>3</sup> /μL | 150 - 450            |
| <b>Comments</b> Manually checked. No clumps noted. Kindly correlate clinically and advise follow up. |              |                     |                      |
| Platelet Hematocrit<br>Calculated                                                                    | <b>0.179</b> | %                   | 0.2 - 0.5            |
| Mean Platelet Volume (MPV)<br>Calculated                                                             | 12.9         | fL                  | 7 - 13               |

- Reference Ranges are in accordance with Dacie & Lewis Practical Hematology International Edition (12th).
- As per International Council for Standardization in Hematology's recommendations Differential Leucocyte counts are additionally reported in Absolute numbers in each cell per unit volume of blood.

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| Test                                | Results             | Units  | Biological Reference |
|-------------------------------------|---------------------|--------|----------------------|
| <b>IMMUNOLOGY</b>                   |                     |        |                      |
| Thyroid Stimulating Hormone (TSH)   | <b><u>4.353</u></b> | μIU/mL | 0.4 - 4.049          |
| Serum, Chemiluminescent Immunoassay |                     |        |                      |

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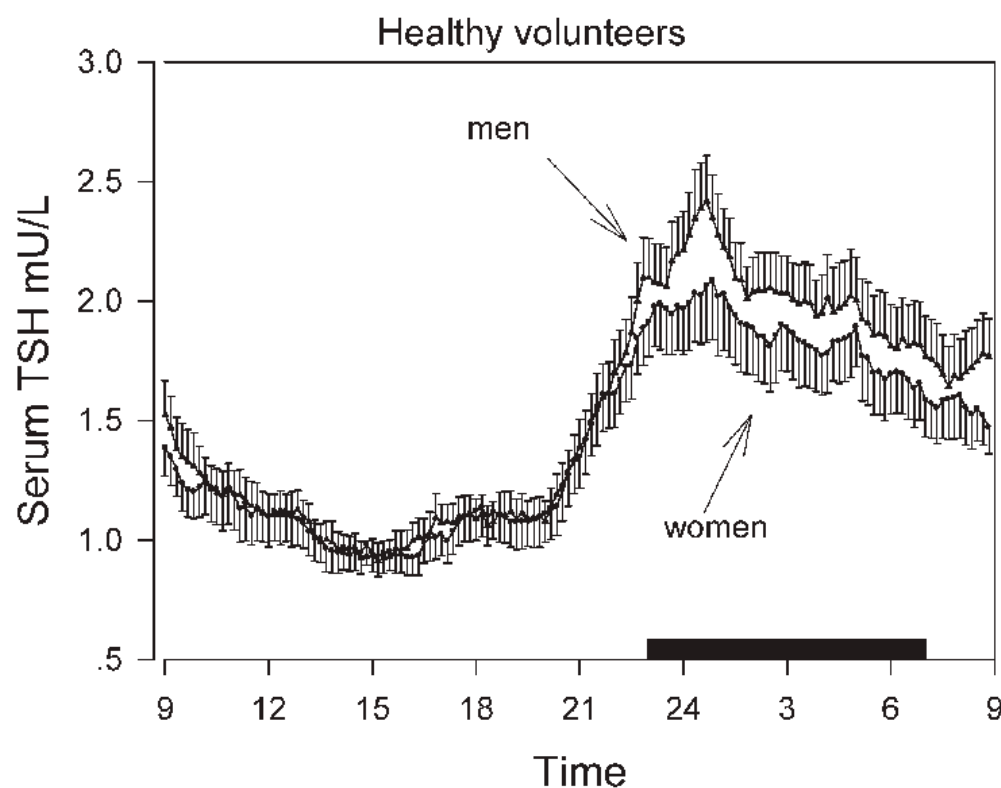


| Test | Results | Units | Biological Reference |
|------|---------|-------|----------------------|
|------|---------|-------|----------------------|

**Clinical Significance:**

Thyroid Stimulating Hormone (TSH), also called Thyrotropin is a hormone secreted into the blood by the Pituitary gland (a gland present in the brain). It signals the thyroid gland to make and release the thyroid hormones (T3 & T4) into the blood. The VITROS TSH3 assay meets the criteria for a third generation assay with a high level of precision and a limit of quantification (LOQ) of 0.01  $\mu$ IU/mL.

High TSH level indicates that the thyroid gland is not making enough thyroid hormone (primary hypothyroidism). Low TSH level usually indicates that the thyroid is producing too much thyroid hormone (hyperthyroidism).



**Factors influencing TSH levels**

- TSH level** shows a significant decline after meal intake in comparison to fasting values. If the patient is taking any thyroid medication different times each day, they may sometimes be taking the thyroid hormone on an empty stomach, and sometimes with or after having food. This may have clinical implications in the diagnosis and management of hypothyroidism, especially Subclinical hypothyroidism.
- Circadian Rhythm:** TSH levels follow a circadian variation, reaching peak levels between Morning 2 - 4 am and at a minimum between Evening 6-10 pm. The above graph considers a sleep window of 11:00 PM to 7:30 AM. The variation is of the order of 50%. There are studies which quote variations up to 70 % depending on when the sample is drawn during which time of the day. Hence time of sample collection during a day can significantly influence on the measured serum TSH concentrations.
- Other Factors:** It is important to recognize that TSH is a labile hormone and is subject to non-thyroidal pituitary influences (glucocorticoids, somatostatin, dopamine etc.), stress, activity, that can disrupt the TSH/FT4 relationship. Genetics, Poisonous substances and radiation exposure, Inflammation of the thyroid gland, Deficiency or excess of iodine in the diet, Pregnancy, Certain medications - antidepressants, cholesterol lowering drugs, chemotherapy drugs, steroids, Thyroid cancer.

In **pregnant females** the reference range of TSH differs. Please refer the table below for the same:-

| PREGNANCY | TSH REFERENCE RANGE ( $\mu$ IU/mL) |
|-----------|------------------------------------|
|-----------|------------------------------------|

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| 1st Trimester | 0.100-2.500 |       |                      |
| 2nd Trimester | 0.200-3.00  |       |                      |
| 3rd Trimester | 0.300-3.00  |       |                      |

#### References:

- Indian Journal of Endocrinology and Metabolism 18(5):p 705-707, Sep-Oct 2014.
- [http://www.pnei-it.com/1/upload/thyrotropin\\_secretion\\_patterns\\_in\\_health\\_and\\_disease.pdf](http://www.pnei-it.com/1/upload/thyrotropin_secretion_patterns_in_health_and_disease.pdf)

**Vitamin B12, Cyanocobalamin**  
Serum, Chemiluminescent Immunoassay

257

pg/mL

239 - 931

#### Clinical Significance:

- Lower value:** Lack of intrinsic factor, total or partial gastrectomy, atrophic gastritis, intrinsic factor antibodies, malabsorption: regional ileitis, resected bowel, tropical sprue, celiac disease, pancreatic insufficiency, bacterial overgrowth & achlorhydria, loss of ingested vitamin B12, fish tapeworm, dietary deficiency: vegetarians, congenital disorders: orotic aciduria & transcobalamin deficiency, increased demand: pregnancy specially last trimester.
- Higher value:** Chronic renal failure, congestive heart failure, acute & chronic myeloid leukemia, polycythemia vera, carcinomas with liver metastasis, liver disease, drug induced cholestasis & protein malnutrition
- An associated increase in **homocysteine levels** is an independent risk marker for cardiovascular disease and deep vein thrombosis.
- HoloTranscobalamin II** levels is a more accurate marker of active Vit B12 component.

**Vitamin D, 25-Hydroxy**  
Serum, Chemiluminescent Immunoassay

33.8

ng/mL

Deficient: < 20  
Insufficient: 20-30  
Sufficient: 30-100  
Toxicity: > 100

#### Clinical Significance:

For the diagnosis of vitamin D deficiency, it is recommended that there be a clinical correlation with serum 25(OH)vitamin D, calcium, parathyroid hormone, and alkaline phosphatase. While monitoring oral vitamin D therapy, serum 25(OH)vitamin D should be tested after 3 months of treatment. However, the required dosage of vitamin D supplements and time to achieve target vitamin D levels show seasonal and individual variability depending on age, body fat, sun exposure, physical activity, genetic factors (especially variable vitamin D receptor responses), associated liver or renal disease, malabsorption syndromes, and calcium or magnesium deficiency influencing the metabolism of vitamin D.

**Insulin, Fasting**  
Serum, Chemiluminescent Immunoassay

19.6

μIU/mL

2.3 - 26

Dayal Prakash Jaiswal

25 Year(s)/Male

Ref. by :

Partner : Elevate now

Report Ref. ID : NOA6055546

Patient ID : OHPXSBGO1914626

Collected : 29/01/2026 08:45 AM

Received : 29/01/2026 11:01 AM

Reported : 29/01/2026 04:09 PM



MC-6357



## Test

## Results

## Units

## Biological Reference

## Clinical Significance:

- Conditions associated with elevated insulin levels include obesity, Cushing's syndrome, acromegaly, insulinoma, administration of contraceptives and early stage of type 2 diabetes.
- Conditions associated with decreased insulin levels are Type 1 diabetes, late stage of type 2 diabetes and chronic pancreatitis.

Approved By

Dr. Adwaita Kapur  
MBBS, MD (Pathology)  
Pathologist

Dayal Prakash Jaiswal

25 Year(s)/Male

Ref. by :

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MC-6357



| Test | Results | Units | Biological Reference |
|------|---------|-------|----------------------|
|------|---------|-------|----------------------|

Reviewed By

Dr. Shivani Tiwari  
MBBS, DCP (Pathology)  
Pathologist

Dr. Ankita Nayak  
MBBS, MD (Pathology)  
Pathologist

## CONDITIONS OF LABORATORY TESTING &amp; REPORTING

- It is presumed that the test sample belongs to the patient named or identified in the test requisition form. Test results released pertain to the specimen submitted.
- Laboratory investigations are only a tool to facilitate arriving at a diagnosis and should be clinically correlated by the Referring Physician.
- All tests are performed and reported as per the turnaround time stated in the Orange Health Labs Directory of Services (DOS).
- Orange Health Labs confirms that all tests have been performed or assayed with the highest quality standards, clinical safety & technical integrity.
- All test results are dependent on the quality of the sample received by the Laboratory and the assay technology.
- Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.
- A requested test might not be performed if:
  - The specimen received is insufficient or inappropriate, or the specimen quality is unsatisfactory
  - Incorrect specimen type
  - Request for testing is withdrawn by the ordering doctor or patient
  - There is a discrepancy between the label on the specimen container and the name on the test requisition form
- Test results may show interlaboratory variations.
- Test results are not valid for medico-legal purposes.
- This is a computer-generated medical diagnostic report that has been validated by an Authorized Medical Practitioner/Doctor. The report does not need a physical signature.

## REPORT PRINTOUT/HARDCOPY

- Patients can collect free report printouts from our Lab or the nearest Collection Centre.
- Hard copies can be delivered within 24 hours for ₹200/report (non-refundable after dispatch).
- For assistance, contact Orange Support at +91-9008111144.